CASE SCENARIO:

Patient Name: PHCG

Age: 92 Year

Gender: Male

Allergy Status: No Known Allergy

Past Medical History: Alzheimer’ s Disease, Old IHD, BPH

Present Illness: Acute CVA – GC bleed extending to right corona radiata with left hemiparesis, Sepsis – Aspiration Pneumonia (Resolving).

Past Medication History:

T. ESCOSPRIN AV (75/10MG) 0-0-1
T. VELTAM S 0-0-1
T. DAPAGLIFLOZIN 5MG 0-1-0
T. CIDMUS 50MG 1/2-0-0
T. CARDIVAS CR 10MG 1-0-0
T. OXRA 5 MG 1-0-0

On 24/09/2026 The Patient Serum Potassium Level Declined to **3.2 mmol/L**

On 25/09/2026 The Patient Serum Potassium Level Declined to **3.2 mmol/L**

On 26/09/2026 The Patient Serum Potassium Level Declined to **3.0 mmol/L**

LABORATORY DATA:

Physical Examination:

Blood Pressure: 110/70 mmHg

Heart Rate: 120 BPM

Respiratory Rate: 22CPM

MANAGEMENT:

On 24/09/2026 She was started with Syp Potklor 10ml TID

On 25/09/2026 She was treated with Inj KCl 2 AMP in 500ml fluids @ 100CC/hrs

The Decreased Serum Potassium Level (Hypokalemia) was resolved on 27/09/2026

TASK 1: ADVERSE DRUG REACTION IDENTIFICATION:

Suspected reaction: Decreased Serum Potassium Level (Hypokalemia)

Suspected drug: INJ. MANNITOL (MANNITOL)

Explanation:

Mannitol (MANNITOL) Increases the Osmotic Pressure in the Renal Tubules, Reducing Water Reabsorption and Increasing Urine Flow. This Increases Distal Sodium Delivery and Tubular Flow, Promoting Potassium Secretion into the Urine, leading to a Decreased Serum Potassium Level (Hypokalemia).

Inj. Mannitol → Increases Osmotic Pressure in Renal Tubules → Reduces Water Reabsorption and Increases Urine Output → Increases Urinary Flow and Distal Delivery of Na^+ → Promotes K^+ Secretion into Urine → Hypokalemia (Decreased Serum K^+ levels).

So, I decided the Suspected Drug INJ. MANNITOL which Induced Hypokalemia (Decreased Serum K^+ levels).

ADR REPORT FORM OF THIS CASE SCENARIO IS ATTACHED HERE:

Version 1.4

SUSPECTED ADVERSE DRUG REACTION REPORTING FORM

For **VOLUNTARY** reporting of ADRs by Healthcare Professionals

INDIAN PHARMACOPOEIA COMMISSION (National Coordination Centre-Pharmacovigilance Programme of India)

Ministry of Health & Family Welfare, Government of India, Sector-23, Raj Nagar, Ghaziabad-201002

PvPI Helpline (Toll Free) :1800-180-3024 (9:00 AM to 5:30 PM, Monday-Friday)

Initial Case ☑ | Follow-up Case ☐

FOR AMC / NCC USE ONLY

Reg. No. / IPD No. / OPD No. / CR No. : 220086009

AMC Report No. :

Worldwide Unique No. :

A. PATIENT INFORMATION *

1. Patient Initials: PHCH
2. Age or date of birth: 90 Years
3. Gender: M ☑ F ☐ Other ☐
4. Weight (in Kg.)

B. SUSPECTED ADVERSE REACTION *

5. Event / Reaction start date (dd/mm/yyyy): 24-09-2026
6. Event / Reaction stop date (dd/mm/yyyy): 27-09-2026
7. Describe Event/Reaction management with details , if any

A Patient was treated with Inj Mannitol for Acute CVA for which the patient developed Decreased Serum Potassium level. (Hypokalemia).

12. Relevant investigations with dates : Serum K+ level : 23-09-2026 - 3.6 mmol/L 27-09-2026 - 3.6 mmol/L 24-09-2026 - 3.2 mmol/L 25-09-2026 - 3.2 mmol/L 26-09-2026 - 3.0 mmol/L

13. Relevant medical / medication history (e.g. allergies, pregnancy, addiction, hepatic, renal dysfunction etc.)

K/C/O : Alzheimer's disease, Old IHD, BPH.
Diag : Acute CVA - GC bleed extending to right corona radiata with left hemiparesis, Sepsis - Aspiration Pneumonia.

14. Seriousness of the reaction : No ☑ If Yes ☐ (*please tick anyone*)

☐Death (dd/mm/yyyy) ☐Congenital-anomaly
☐Life threatening ☐Disability
☐Hospitalization-Initial/Prolonged ☐Other Medically important

15. Outcome:

☑Recovered ☐Recovering ☐Not Recovered
☐Fatal ☐Recovered with sequelae ☐Unknown

C. SUSPECTED MEDICATION(S) *

S. No.	B. Name (Brand/ Generic)	Manufacturer (if known)	Batch No. / Lot No.	Expiry Date (if known)	Dose	Route	Frequency	Therapy Dates: Date Started	Therapy Dates: Date Stopped	Indication	Causality Assessment
i	Inj Mannitol (Mannitol)				100ml	IV	BD	22-09-	25-09	Acute CVA	Probable
ii											
iii											
iv#											

9. Action taken after reaction (*please tick*)

S. No. as per C	Drug withdrawn	Dose increased	Dose reduced	Dose not changed	Not applicable	Unknown
i	✓					
ii						
iii						
iv						

10. Reaction reappeared after reintroduction of suspected medication (*please tick*)

Yes	No	Effect unknown	Dose (if re-introduced)

11. Concomitant medical product including self-medication and herbal remedies with therapy dates (Exclude those used to treat reaction)

S. No.	Name (Brand / Generic)	Dose	Route	Frequency (OD, BD, etc.)	Therapy Dates: Date Started	Therapy Dates: Date Stopped	Indication
i	Neb Duolin (Levosalbutamol 500mcg + Ipratropium)	1.25mg +	Mask	Q8hrs	19-09-2026	10-10-2026	Sepsis - Aspiration Pneumonia.
ii							
iii#							

Additional Information :

On 24-09-2026 - Syp Potklor 10ml TID given.

On 25-09-2026 - Inj KCl @ Amp in 500ml fluids @ 100cc/hr.

D. REPORTER DETAILS *

16. Name & Address : Harshitha FMMCH

Pin : 575018 Email : acharyaharshitha38@gmail.com

Contact No- : 8861203149

Occupation : Pharm D Intern Signature : Harshitha

17. Date of this report (dd/mm/yyyy) : 29-09-2026

Signature and Name of Receiving Personnel :

Confidentiality : The patient's identity is held in strict confidence and protected to the fullest extent. Submission of a report does not constitute an admission that medical personnel or manufacturer or the product caused or contributed to the reaction. Submission of an ADR report does not have any legal implication on the reporter.

Use separate page for more information

* Mandatory Fields for suspected ADR Reporting Form